

Elderly Psychosis and Agitation



Section I: Scenario Demographics

Scenario Title:	Elderly Psychosis and Agitation
Date of Development:	Original 01/12/2015, revised 02/28/2018
Target Learning Group:	☐ Juniors (PGY 1 – 2) ☒ Seniors (PGY ≥ 3) ☐ All Groups Recommend joint learning case with hospital security. Include security as learners (or confederates, if need be)

Section II: Scenario Developers

Scenario Developer(s):	Drs. Nicole Kester-Greene and Jen Riley
Affiliations/Institution(s):	University of Toronto, Sunnybrook (NKG), St Mike's (JR)
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Section III: Curriculum Integration

Learning Goals & Objectives	
Educational Goal:	To allow learners to become more comfortable managing severe agitation requiring chemical sedation
CRM Objectives:	1. Demonstrate leadership skills by maintaining control of a loud, chaotic environment and coordinating efforts of this inter-professional team. 2. Communicate effectively with team regarding medication, dose and route of administration. 3. Maintain situational awareness in a chaotic environment to maintain safety of patient, staff and self during application of physical and chemical restraints. 4. Security team objective: Demonstrate safe application of physical restraints while ensuring safety of self, patient and staff.
Medical Objectives:	1. Recognize the need for chemical sedation when conservative measures are ineffective. 2. Use appropriate agents and dosing of sedation agent(s) in an elderly patient. 3. Initiate appropriate work-up of new agitation/psychosis in an elderly patient.

Case Summary: Brief Summary of Case Progression and Major Events

A 68-year old woman is found at a bus stop exhibiting bizarre behaviour. She is brought to the ED by paramedics. In the ED, she is expressing paranoid delusions. Her agitation escalates and does not respond to verbal de-escalation or an overwhelming show of force. She will require physical and chemical sedation to facilitate the work-up for her new onset psychosis.

Note: This case may involve physical restraint. Both the standardized patient and sim participants should be briefed on the use of simulated restraint. It is essential that a safe word like "Time out" is pre-determined in case any participants are feeling unsafe at any point in the case. This would immediately halt the case. A detailed pre-briefing regarding safety and an extremely clear explanation of this safe word are paramount. Similarly, instructors must be watching closely for safety and cut the scenario/intervene quickly if they feel anyone may be harmed.



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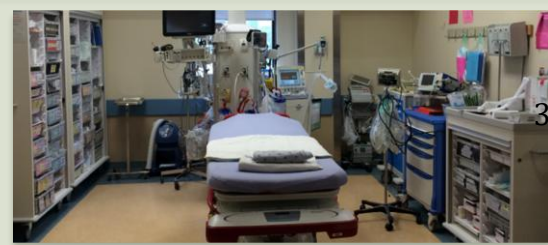


Section IV: Scenario Script

A. Scenario Cast & Realism			
Patient:	☐ Computerized Mannequin ☐ Mannequin ☒ Standardized Patient ☒ Hybrid ☐ Task Trainer	Realism: <i>Select most important dimension(s)</i>	☒ Conceptual ☐ Physical ☒ Emotional/Experiential ☐ Other: ☐ N/A
Confederates			
Brief Description of Role			
Patient	Talking incessantly about her neighbour putting poison in her coffee and cameras in her apartment. She states that he records her every move. If anyone tries to obtain further history she becomes paranoid of the medical staff and starts accusing them of collaborating with her neighbour. Looks around room at nothing in particular due to visual hallucinations. Tries to hit anyone who approaches her for vital signs, IV access, etc. Over 3-5 minutes, escalates verbally (yelling) and physically (hitting bed, walls) until given chemical sedation.		
ED RN	Acts as ED RN. Assists with bedside care. Responsible for mocking up administration of IM sedation in order to ensure SP is not harmed or given any injections.		
Security	(Optional). Acts as hospital security. Assists with physical restraint.		
B. Required Monitors			
☒ EKG Leads/Wires	☐ Temperature Probe	☐ Central Venous Line	
☒ NIBP Cuff	☐ Defibrillator Pads	☐ Capnography	
☒ Pulse Oximeter	☐ Arterial Line	☐ Other:	
C. Required Equipment			
☒ Gloves	☒ Nasal Prongs	☐ Scalpel	
☒ Stethoscope	☒ Venturi Mask	☐ Tube Thoracostomy Kit	
☐ Defibrillator	☒ Non-Rebreather Mask	☐ Cricothyroidotomy Kit	
☒ IV Bags/Lines	☒ Bag Valve Mask	☐ Thoracotomy Kit	
☒ IV Push Medications	☒ Laryngoscope	☐ Central Line Kit	
☐ PO Tabs	☐ Video Assisted Laryngoscope	☐ Arterial Line Kit	
☐ Blood Products	☒ ET Tubes	☐ Other:	
☐ Intraosseous Set-up	☐ LMA	☐ Other:	
D. Moulage			
Standardized patient should look unkempt.			
E. Approximate Timing			
Set-Up:	10-15 min	Scenario:	10 min
		Debriefing:	15 min



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Section V: Patient Data and Baseline State

A. Clinical Vignette: To Read Aloud at Beginning of Case

The charge nurse comes to you: "There is a 68 year old woman in the seclusion room. She was observed pacing and acting bizarre at the bus stop. EMS managed to talk her into ambulance. On route she told them her neighbour is trying to poison her. Initially, she was calm but now she is starting to get agitated. She doesn't have any previous psych admissions in the system. We couldn't get any vital signs."

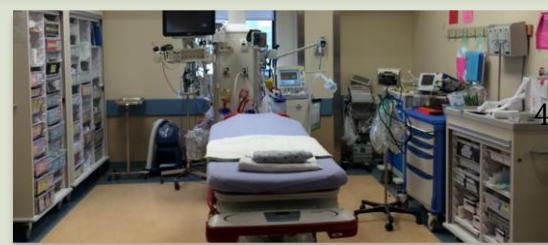
B. Patient Profile and History

Patient Name: Amina Patel		Age: 68		Weight: 70 kg	
Gender: ☐ M ☒ F		Code Status: Presumed Full			
Chief Complaint: Bizarre behaviour					
History of Presenting Illness: Seen at bus stop acting strangely. Bystander called EMS.					
Past Medical History:	Hypertension		Medications:	Hydrochlorothiazide	
	Osteoporosis			Alendronate	
Allergies: Unknown					
Social History: Unknown. Unable to reach emergency contacts.					
Family History: Unknown.					
Review of Systems:	CNS:	Denies any issues.			
	HEENT:	Denies any issues.			
	CVS:	Denies any issues.			
	RESP:	Denies any issues.			
	GI:	Denies any issues.			
	GU:	Denies any issues.			
	MSK:	Denies any issues.		INT:	Denies any issues.

C. Baseline Simulator State and Physical Exam

☒ No Monitor Display		☐ Monitor On, no data displayed		☐ Monitor on Standard Display	
HR: /min	BP:	RR: /min	O ₂ SAT: %		
Rhythm:	T: °C	Glucose: mmol/L	GCS: 14 (E4 V4 M6)		
General Status: Agitated and paranoid.					
CNS:	Responding to visual hallucinations. No obvious neurological deficits.				
HEENT:	Unable to assess.				
CVS:	Unable to assess.				
RESP:	Unable to assess.				
ABDO:	Unable to assess.				
GU:	Unable to assess.				
MSK:	Unable to assess.		SKIN:	Unable to assess.	

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Section VI: Scenario Progression

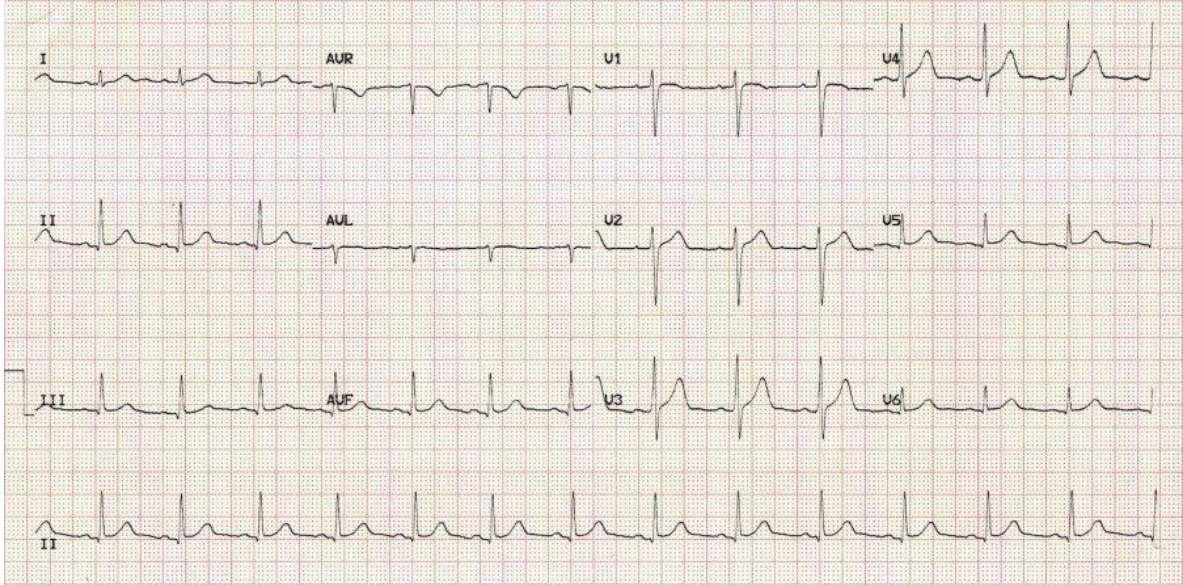
Scenario States, Modifiers and Triggers			
Patient State	Patient Status	Learner Actions, Modifiers & Triggers to Move to Next State	
1. Baseline State Unable to obtain vital signs	Agitated & paranoid. Pacing room. Will not allow RN assessment or monitors.	<u>Learner Actions</u> - ☐ Attempt verbal de-escalation - ☐ Call for help (security, SW) - ☐ Establish plan for sedation with team - ☐ Attempt a show of force - ☐ Consider team safety - ☐ IM sedation (<i>Appropriate dose: ketamine 3-5mg/kg = 200-300mg OR lorazepam 2mg +/- 2-5mg Haldol/olanzapine</i>)	<u>Modifiers</u> <i>Changes to patient condition based on learner action</i> - 5 minutes → RN to suggest "Should we use chemical sedation?" - If still not sedation 2 min after prompt, RN to state "This is dangerous agitation and we need to control it more effectively." <u>Triggers</u> <i>For progression to next state</i> - Appropriate dose sedation → 2. Sedated - Large dose sedation → 3. Over-sedated
2. Sedated Rhythm: Sinus tach HR: 105/min BP: 110/70 RR: 12/min O ₂ SAT: 90% T: 36.3°C	<i>Exchange standardized pt for manikin.</i> Patient drowsy, GCS 11 (E3 M5 V4)	<u>Learner Actions</u> - ☐ IV access - ☐ Monitors, O ₂ - ☐ Physical Exam - ☐ CBG - ☐ BW: Tox, Sepsis - ☐ IV fluid bolus - ☐ EKG - ☐ Soft restraints	<u>Modifiers</u> - IV fluids given, HR → 90 - O ₂ given, sats → 96% <u>Triggers</u> - All actions complete → 5. Resolution
3. Over-sedated Rhythm: Sinus HR: 90/min BP: 95/55 RR: 6/min O ₂ SAT: 80% T: 36.3°C	<i>Exchange standardized pt for manikin.</i> Patient non-responsive, but has a pulse.	<u>Learner Actions</u> - ☐ IV access - ☐ Monitors, O ₂ - ☐ CBG - ☐ Intubation	<u>Modifiers</u> - O ₂ applied → O ₂ SAT 85% <u>Triggers</u> - Intubation → 4. Intubated
4. Intubated Rhythm: Sinus HR: 80/min BP: 105/65 RR: 12/min O ₂ SAT: 99%	Patient intubated and sedated.	<u>Learner Actions</u> - ☐ Physical Exam - ☐ BW: Tox, Sepsis - ☐ IV fluid bolus - ☐ EKG - ☐ CT head - ☐ Sedation - ☐ Post-intubation CXR	<u>Modifiers</u> <u>Triggers</u> - All actions complete → 5. Resolution
5. Resolution Unchanged		<u>Learner Actions</u> - ☐ Foley, UA - ☐ Head CT - ☐ Consider LP - ☐ Consult Medicine or ICU	<u>Triggers</u> - Head CT → End of Case

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Section VII: Supporting Documents, Laboratory Results, & Multimedia

Laboratory Results
None given during case.

Images (ECGs, CXRs, etc.)
EKG – Sinus rhythm, normal QTc


From: <https://lifeinthefastlane.com/ecg-library/normal-sinus-rhythm/>

Ultrasound Video Files (if applicable)
None available

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Section VIII: Debriefing Guide

General Debriefing Plan	
☐ Individual	☒ Group
☐ With Video	☒ Without Video
Objectives	
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Sample Questions for Debriefing	
<u>Medical Expert</u> 1. Name some sedation agents for use in severe, acute agitation? How you choose agent, dose and route of administration? Contraindications for each agent? 2. What are the indications for chemical restraint? 3. What is your differential diagnosis of new psychosis? Differences between a 22-year old and 72-year old? 4. How can you distinguish organic psychosis from primary psychiatric psychosis? <u>CRM</u> 1. Describe a stepwise approach to the acutely agitated patient? 2. How can you organize a team to perform safe physical restraint? <u>Systems</u> 1. What are some <u>systems level interventions</u> that enhance safety in clinical encounters with agitated patients?	
Key Moments	
Attempting conservative management of agitation.	
Recognizing the need for chemical sedation.	
Choosing a sedation agent for elderly patient.	
Initiating a work-up for new onset psychosis.	

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References

- Marx, J. A., Hockberger, R. S., Walls, R. M., & Adams, J. (2013). *Rosen's emergency medicine: Concepts and clinical practice*. St. Louis: Mosby.
- Nickson, C. (2014). *Chemical Restraint*. Life in the Fast Lane. Retrieved from <https://lifeinthefastlane.com/ccr/chemical-restraint/>
- Strayer, R. (2016). *Emergency Management of the Agitated Patient*. Emergency Medicine Updates. Retrieved from <http://emupdates.com/2016/09/03/emergency-management-of-the-agitated-patient/>

